

University Cancer Institute
123 Health Sciences Dr.
Metropolis, USA 12345

ATTN: Pre-Authorization Department, ProCare Health Insurance

DATE: 15-Oct-2023

RE: Letter of Medical Necessity for Venetoclax (Venclexta)

PATIENT: Donna Nelson

MRN: AML029

DOB: 19-Apr-1951

Dear Sir/Madam,

I am writing to request urgent pre-authorization for **Venetoclax (Venclexta)** for my patient, Ms. Donna Nelson, for the treatment of her relapsed Acute Myeloid Leukemia (AML). This treatment is medically necessary and represents the best available therapeutic option for her.

Ms. Nelson is a 72-year-old female who was diagnosed with AML in October 2022 (28th). Her diagnostic bone marrow biopsy showed 55% blasts with a normal karyotype and wild-type molecular status, placing her in the ELN 2022 **intermediate-risk category**.

Due to her age and a performance status of ECOG 2 at diagnosis, she was not considered a candidate for intensive induction chemotherapy. The standard of care for such a patient is a hypomethylating agent. She was therefore started on first-line therapy with **Azacitidine** on November 7, 2022. She initially responded well, with an improvement in her blood counts and a reduction in bone marrow blasts to 10% after four cycles.

Unfortunately, after eleven months of therapy, she now has clear evidence of disease progression. Her most recent labs show worsening pancytopenia and circulating blasts. A bone marrow biopsy performed on October 10, 2023, confirmed progressive disease, with marrow blasts now increased to 45%. This demonstrates that her disease has relapsed and is now refractory to single-agent hypomethylating therapy.

Without further treatment, her prognosis is exceptionally poor, measured in weeks to a few months.

The current NCCN guidelines (v3.2023) strongly recommend the combination of Venetoclax with a hypomethylating agent for older or unfit adults with AML, both in the frontline and relapsed/refractory settings. The VIALE-A trial, published in the *New England Journal of Medicine*, demonstrated a significant survival benefit for the combination of Venetoclax and Azacitidine over Azacitidine alone in the frontline setting. While Ms. Nelson has progressed on Azacitidine, the addition of Venetoclax introduces an entirely new mechanism of action (BCL-2 inhibition) and offers a substantial chance of achieving another remission. There is no other approved therapy that offers a comparable chance of response in this clinical scenario.

Therefore, I am requesting authorization for the following regimen:

- **Venetoclax (Venclexta):** 400 mg PO daily, with initial ramp-up.
- **Azacitidine:** 75 mg/m² IV/SC for 7 days each 28-day cycle.

This treatment is critical to control Ms. Nelson's aggressive leukemia, improve her quality of life, and extend her survival. A delay in initiating this therapy will be detrimental to her outcome.

Please find attached her relevant clinic notes, pathology reports, and lab results. Do not hesitate to contact my office if further information is required.

Sincerely,

Dr. Eleanor Vance, MD
Hematology-Oncology
NPI: 1234567890